جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : سنيه فاروق ابوالعلا سيد
المحافظة : الجيزه **الحي / المدينة :** امبابه **العنوان :** --
رقم البطاقة : 26603110101486 **رقم التليفون :** 01063040957 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 11-03-1966 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سنيه فاروق ابوالعلا سيد
National ID: 26603110101486

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bilateral mammogram and breast ultrasound**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

GUSTAVE ROUSSY
INTERNATIONAL EGYPT

Patient Name	: الأستاذه / سنيه فاروق ابو العلا سيد	Request No.	: 13005863
Patient No.	: 26603110101486	Request Date	: 14/04/2026
Path No.	: 1001/26	Verification Date	: 16/04/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 60 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left thyroid lobe nodule (TIRADs 4) in patient with history of treated breast cancer.
Nature of specimen: Tru-cut biopsy, collected on 14/04/2026, at 02:20 pm, cold ischemia time: 20 seconds.

Gross Examination:
Multiple fragmented grey tan tissue cores, ranging in size between 0.1 cm & 0.3 cm long. Totally submitted.

Microscopic Examination:
Histopathological examination reveals fragmented cores of thyroid tissue formed of variable size follicles (mostly microfollicles) lined by bland follicular cells & containing colloid. No remarkable inflammation. No nuclear anaplasia or papillary features within the limits of the small fragmented biopsy received.

Diagnosis:
Left thyroid lobe nodule, Tru-cut biopsy:
- Findings are in favor of follicular thyroid lesion. No evidence of nuclear features within biopsy received.
** Comment: The limitation of core biopsy in sampling is not informative if this lesion is follicular adenoma or carcinoma as it depends on vascular and capsular invasion which requires extensive sampling and thorough examination.

Signature
Dr. Walaa Ibrahem Ghoneim.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]



National ID - Front
[[DOCTYPE:ID_FRONT]]

Name: سنيه فاروق ابوالعلا سيد **MR. :** 6085	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient underwent a bone scan on 9 February 2026 which demonstrated an equivocal tracer uptake pattern at the lower end of the sacrum/coccyx as well as the left sacroiliac articulation; magnetic resonance imaging of the bony pelvis was recommended and the patient was advised to continue letrozole (Femara).

The patient is a 60-year-old individual with a left breast mass, clinical stage cT2N1, estrogen receptor 6/8, progesterone receptor 6/8, HER2 + 3, and Ki-67 15 %. Six cycles of neoadjuvant TCHP were completed, followed by left wide local excision and sentinel lymph node biopsy on 15 March 2023, resulting in pathology pT0N0/11 with a pathologic complete response. Adjuvant radiotherapy was completed, and letrozole therapy was initiated in March 2023. The bone scan performed on 9 February 2026 showed the same equivocal sacral and left sacroiliac findings; the patient has been on letrozole for three months and continues follow-up.

The patient has a solitary bony lesion and will continue letrozole together with zoledronic acid (Zometa) for three months; follow-up computed tomography scans are planned due to the presence of tiny pulmonary nodules.

The patient experienced trauma under the eye and declined emergency department or ophthalmology evaluation; she remains fully conscious and oriented to time, person, and place, and denies any head injury. Zoledronic acid will be continued after laboratory assessment, and letrozole will be administered as 1 tablet × 1 × 90 days with supportive care also prescribed for 1 tablet × 1 × 90 days.

The patient has a known psychiatric history of schizophrenia and epilepsy, which contributes to an unstable gait. She also underwent total thyroidectomy for a goitre; pathology showed no malignancy. Thyroid function tests are being monitored, and the patient will continue letrozole, supportive care, and follow-up chest-abdomen-pelvis CT imaging.

تحريرا في ,

التاريخ :	20-09-2026	**الوقت :**	

الطبيب المعالج : Ali Adel

يعتمد ,

National ID - Back
[[DOCTYPE:ID_BACK]]

05/12/2026

MDT FORM

Name:	Sania Farouk . Said	**MRN:**	6085
Age:	60 Y	**ID:**	26503110101486
Menopausal Status:		**Phone:**	01272712171
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:	BREAST	**Reason Of MDT:**	THYROID

Clinical Data:

BX: Diagnosis : Left thyroid lobe nodule, Tru-cut biopsy

ER: **PR:** **HER2** **KI67:**

Sonomammography:

Large left thyroid mass measuring 3 x3.5 cm showing isoechoic pattern with central cystic degeneration totally concealing the left thyroid lobe.

Normal size of right thyroid lobe and ismuth, the right lobe showing multi-lobulated

Mets Workup:

Equivocal tracer uptake pattern at the lower end of the sacrum/coccyx as well as left sacroiliac articulation, for regional radiological correlation.
Few (Two to three) right lung tiny scattered pulmonary nodules for follow up.
Normal CT scan of the abdomen and pelvis.

No hepatic metastasis.

History:

Recommendations:

FOR THYRODECTOMY

MDT Outcome & Plan

For Total thyroidectomy +/- LN dissection (after new neck US)

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ خالد عبد العزيز	د/ محمد الدراوي				

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: الأستاذه / سنيه فاروق ابو العلا سيد	Request No.	: 13008841
Patient No.	: 26603110101486	Request Date	: 29/08/2022
Path No.	: 2519/22	Verification Date	: 15/09/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 56 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass lesion

Nature of the specimen: Tru cut biopsy collected 29-8-2022 (12:40 PM).

Gross Examination:
Three greyish white tissue cores from 0.2 cm to 1.5 cm . Totally submitted .
Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:
- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Ductal carcinoma in situ: No

ER: Moderate nuclear reaction in 50% of tumor cells (Allred score 6/8)
PR: Moderate nuclear reaction in 50% of tumor cells (Allred score 6/8)
Her-2: strong complete membrane reaction in >10% of tumor cells (score 3+)
Ki 67: 15 % nuclear reaction

Diagnosis:
Left breast mass lesion , Tru cut biopsy:
- Invasive duct carcinoma grade II

ER : Positive (6/8)
PR : Positive (6/8)
HER2 : Positive (score 3+)
Ki67: 15%

Verified by :
Dr.Marwa mahmoud

Room No. (6) - First Floor-Building No. 88 - Nile Corniche - Sitio El Qudema- Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Name : Sonia Farouk Abul Ela Said
Branch : Dokki

Date : 09-02-2026 10:46
Medical No. : 312894

Indication: 59-year-old female with history of breast carcinoma. Evaluation for possible osseous metastatic disease.
Radioisotope: Tc-99m MDP
Imaging Protocol: Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection.

FINDINGS:

- Active tracer uptake at the lower end of the sacrum/coccyx as well as left sacroiliac articulation, for further assessment.
- No other lesions detected.
- Degenerative/Arthritic changes are seen at different levels of the spine, sternum, both shoulders, as well as the weight-bearing joints.

IMPRESSION:

- Equivocal tracer uptake pattern at the lower end of the sacrum/coccyx as well as left sacroiliac articulation, for regional radiological correlation.
- Follow up is recommended.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name	: الأستاذة / مانيه فاروق ابو العلا سيد	Request No.	: 13000065
Patient No.	: 26603110101486	Request Date	: 15/03/2023
Path No.	: 820/2023	Verification Date	: 05/04/2023
Sex	: Female	Financial Category	: ام دار السلام - تلقة الدولة
Age	: 57 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed y

Pathology Report

Clinical Data :
Left breast cancer post NACT.

Nature of the specimen :
left WLE specimen and SLNs biopsy collected at 15/3/23.

Gross Examination:
-Left WLE specimen measured 8x7x1.5 cm , with covering skin elipse measured 5.5x1.5 cm
- Tumor extent : An ill defined rubbery greyish white tumour bed site (wired and clipped0
- Size: 1.5x1 cm
- Skin involvement: unremarkable grossly
- Distance from margins:1 cm away fromsuperior and inferior margins
SLNs: Fibrofatty tissue peice measured 4.5x3.5 cm enclosing 11 nodes up to 1.5x1 cm ,
-Extra deep margin :Muscular tissue peice measured 5x1 cm , free of gross indurations
-Extra inferior margin :Fibrofatty tissue peice measured 9x4 cm , free of gross indurations.
- SLNs: Free SLNs (0/7).
Other lymph nodes left for paraffin.

Microscopic examination:
- Histological type: Total tumor regression in a previously diagnosed carcinoma.
- Lymph node involvement: Free SLNs (0/11)
- Resection Margins: Free resected margins and extra margns
- Histologic response to therapy: Pathologic complete response.

Diagnosis:
Right Breast cancer , post NACT ,WLE and specimens :
-Total tumor regression in a previously diagnosed invasive carcinoma
-Free resected margins and extra margins

Verified by :
DR.Menna Nabil

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذة / مارية فاروق ابو السلام سيد	Request No.	: 13008841
Patient No.	: 26603110101486	Request Date	: 29/08/2022
Path No.	: 2519/22	Verification Date	: 05/09/2022
Sex	: Female	Financial Category	: ر السلام - تكلة الدولة
Age	: 56 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohame

Pathology Report

Clinical Data:
Left breast mass lesion
Nature of the specimen:
Tru cut biopsy collected 29-8-2022 (12:40 PM)
Gross Examination:
Three greyish white tissue cores from 0.2 cm to 1.5 cm . Totally submitted .

Microscopic examination:
- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Ductal carcinoma in situ: No

Diagnosis:
Left breast mass lesion , Tru cut biopsy:
- Invasive duct carcinoma grade II
- For IPT (ER- PR -HER2 -KI67)

Prof.Dr.Gamal Abdelati
Dr.Israa Elgendy

Verified by :
DR.Israa Elgendy

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: السيدة / سنيه فاروق ابو العلا سيد	Request No.	: 13008841
Patient No.	: 26603110101486	Request Date	: 29/08/2022
Path No.	: 2519/22	Verification Date	: 15/09/2022
Sex	: Female	Financial Category	: اعلام - نفقة الدولة
Age	: 56 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohar

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Left breast mass lesion , Tru cut biopsy:
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ER : Positive (6/8)
PR : Positive (6/8)
HER2 : Positive (score 3+)
Ki67: 15%

Verified by :
Dr.Marwa mahmoud

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	: الأستاذة / سامية فاروق ابو العلا سيد	Request No.	: 13000065
Patient No.	: 26603110101486	Request Date	: 15/03/2023
Path No.	: 820/2023	Verification Date	: 05/04/2023
Sex	: Female	Financial Category	: برنامج دار السلام - نفقة الدولة
Age	: 57 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed yo

Pathology Report

-Free SI.Ns (0/11)
-Pathologic complete response
-Stage : ypT0 , ypN0

Prof.Gamal Abdelatty
Dr.Wagdy Fadl

Verified by :
DR.Menna Nabil

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
الهيئة العامة للمراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Saneya Farouk Abu Al Ela Sayed	Date : 03/11/2024
Exam : C.T.BBRAIN WITH CONTRAST	Age : 58 Year(s)
Medical ID: 3381/2022	Pat. ID: 18047

MULTI-SLICE CT SCAN OF THE BRAIN AFTER IV-CONTRAST ADMINISTRATION: -

Findings:

- Preserved normal gray-white matter interface.
- No focal cerebral areas of abnormal enhancement pattern.
- No extra-axial or intra-cerebral areas of fresh blood densities.
- Unremarkable cerebral ventricles with no midline shift or deformity.
- Prominent cortical sulci and extra axial CSF spaces.
- No midline shift.
- Unremarkable posterior fossa structures.

OPINION:

- **Unremarkable contrast enhanced CT scan of the brain.**

DR.mona ali

Prof. Menna Tallah Hanafy, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]